

TRINOLONE CREAM 0.1% W/W

Composition:
Triamcinolone Acetonide 0.1% w/w

Product Description:
A white and smooth cream.

Pharmacodynamics:
Triamcinolone is a synthetic glucocorticoid and has potent anti-inflammatory, hormonal and metabolic effects. It is essentially devoid of mineralocorticoid activity when administered in therapeutic doses. The mechanism of anti-inflammatory activity of topical corticosteroids is unclear.

Pharmacokinetics:
The extent of percutaneous absorption of topical corticosteroids is determined by many factors including the vehicle, the integrity of the epidermal barrier, and the use of occlusive dressings. Topical corticosteroids can be absorbed from normal intact skin. Inflammation and/or other disease processes in the skin increase percutaneous absorption. Occlusive dressings substantially increase the percutaneous absorption of topical corticosteroids. Once absorbed through the skin, topical corticosteroids are handled through pharmacokinetic pathways similar to the systemically administered corticosteroids. Corticosteroids are bound to plasma proteins in varying degrees. Corticosteroids are metabolized primarily in the liver and are then excreted by kidneys. Some of the topical corticosteroids and metabolites are also excreted into the bile.

Indication:
Treatment of acute or chronic inflammatory dermatoses such as localized neurodermatitis, psoriasis, atopic or seborrheic dermatitis, the inflammatory phase of xerosis and anogenital pruritus.

Recommended Dosage:
A thin film of the cream is applied to the affected skin one to three times a day. Dosage of once or twice a day is often effective.

Route of Administration: Topical

Contraindications:
It is contraindicated in patients who are hypersensitive to triamcinolone or other corticosteroids.

Warnings and Precautions:
Systemic absorption of topical corticosteroids has produced reversible hypothalamic-pituitary-adrenal (HPA) axis suppression, manifestations of Cushing's syndrome, hyperglycemia and glycosuria in some patients. Conditions which augment systemic absorption include the application of the more potent steroids, use over large surface areas, prolonged use, and the addition of occlusive dressings. Therefore, patients receiving a large dose of a potent topical steroid applied to a large surface area or under an occlusive dressing should be evaluated periodically for evidence of HPA axis suppression by using the urinary free cortisol and ACTH stimulation tests. If HPA axis suppression is noted, an attempt should be made to withdraw the drug, to reduce the frequency of application, or to substitute a less potent steroid. Recovery of HPA axis function is generally prompt and complete upon discontinuation of the drug. Infrequently, signs and symptoms of steroid withdrawal may occur, requiring supplemental systemic corticosteroids. If irritation develops, topical corticosteroids should be discontinued and appropriate therapy instituted.

In the presence of dermatological infections, the use of an appropriate antifungal or antibacterial agent should be instituted. If a favourable response does not occur promptly, the corticosteroids should be discontinued until the infection has been adequately controlled. Paediatric patients may demonstrate greater susceptibility to topical corticosteroid-induced HPA axis suppression and Cushing's syndrome than mature patients because of a larger skin surface area to body weight ratio. HPA axis suppression, Cushing's syndrome, and intracranial hypertension have been reported in children receiving topical corticosteroids. Manifestations of adrenal suppression in children include linear growth retardation, delayed weight gain, low plasma cortisol levels, and absence of response to ACTH stimulation. Manifestations of intracranial hypertension include bulging fontanelles, headaches and bilateral papilledema. Administration of topical corticosteroids to children should be limited to the least amount compatible with an effective therapeutic regimen. Chronic corticosteroid therapy may interfere with the growth and development of children.

Interactions with Other Medicaments:
No known drug interaction.

Pregnancy and Lactation:
Pregnant women and nursing mothers should seek the advice of the physician before using this product.

Side Effects:
The side effects are itching, burning, stinging, dryness, folliculitis, hypertrichosis, acneiform eruption, hypopigmentation, perioral dermatitis, allergic contact dermatitis, maceration of the skin, secondary infection, skin atrophy, striae, miliaria.

Symptoms and Treatment of Overdose:
Topically applied corticosteroids can be absorbed in sufficient amount to produce systemic effect. (See Warnings and Precautions).

Effects on Ability to Drive and Use Machine: None known.

Storage Conditions: Store at or below 30°C. Protect from light.

Pack Size: A tube of 15g and 30g.
A box of 50 x 15g / tube.
Pack Size (export only): A jar of 400g.

Shelf-life: 3 years

FURTHER INFORMATION CONCERNING THIS DRUG CAN BE OBTAINED FROM YOUR FAMILY PHYSICIAN/LOCAL GENERAL PRACTITIONER/PHARMACIST.

Manufacturer & Product Registration Holder:
Sunward Pharmaceutical Sdn. Bhd.
No. 9, 11 & 17, Jalan Kempas 4,
Taman Perindustrian Tampoi Indah,
81200 Johor Bahru, Johor, Malaysia.

EL226A-R8
Revision Date : 19/12/2022